

164mm

153mm

additional measures unnecessary. However, if additional correction of severe hypotension is required, administration of an intravenous alpha – adrenergic agonist such as methoxamine or phenylephrine may be considered; epinephrine should be avoided since it aggravates the shock – like reaction.

The administration of oxygen, with assisted respiration may be necessary in the severe poisoning, if methaemoglobinaemia occurs give methylene blue 1 mg per kg of body weight by slow intravenous injection. The circulation may be maintained with infusions of plasma or suitable electrolyte solutions. In the case of severe poisoning with tablets the stomach should be emptied by lavage.

STORAGE CONDITIONS, USER INSTRUCTIONS & PHARMACEUTICAL PRECAUTIONS:

Store below 30°C, in a well closed container.

User Instructions:

Avoid alcoholic beverages.

Pharmaceuticals Precautions:

Loss of potency is accelerated by exposure to heat and moisture.

PACKING / PACK SIZES:

Blister pack of 10 tablets, 10 x 10's/ box and 100 x 10's/ box.

SHELF LIFE:

3 years

MANUFACTURER AND PRODUCT REGISTRATION HOLDER:

SM PHARMACEUTICALS SDN BHD (218620 – M)
Lot 88, Sungai Petani Industrial Estate
08000 Sungai Petani,
Kedah Darulaman,
Malaysia

10.02.2023

SM PHARMACEUTICALS SDN BHD

**SOLBID TABLET 10 MG
(ISOSORBIDE DINITRATE 10 MG)**

DESCRIPTION:

Yellowish Orange, clean, dry, uniform capsule shaped with flat surface and breakline; uncoated tablets.

ACTIONS AND MODE OR MECHANISMS OF ACTION:

Antianginal or cardiac load – reducing agent – Not specifically known but thought to cause a reduction of myocardial oxygen demand. This is attributed to a reduction in left ventricular preload and afterload because of venous (predominantly) and arterial dilation with a more efficient redistribution of blood flow within the myocardium.

PHARMACOLOGY:

Protein Binding: Moderate

Metabolism: Hepatic (very rapid and nearly complete) and in blood (enzymatically) Oral dosage form undergoes extensive first – pass metabolism.

Onset of action:

Note: Although studies have not been done, buccal pharmacokinetics is probably similar to those after sublingual administration.

Oral tablets: 15 to 40 minutes.

Chewable tablets: 2 to 5 minutes.

Extended – release tablets: 30 minutes.

Sublingual tablets: 2 to 5 minutes.

Duration of action:

Note: Although studies have not been done, buccal pharmacokinetics is probably similar to those after sublingual administration.

Oral tablets: 4 to 6 hours.

Chewable tablets: 1 to 2 hours.

Extended – release tablets: 12 hours.

Sublingual tablets: 1 to 2 hours.

Excretion: Renal (after nearly total metabolism)



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INDICATIONS:

Isosorbide dinitrate is used prophylactically in the treatment of angina pectoris.

CONTRAINDICATIONS:

It is contraindicated in patients who have shown purported hypersensitivity or idiosyncrasy to it or other nitrates or nitrites.

SIDE EFFECTS / ADVERSE REACTIONS:

Isosorbide dinitrate may cause flushing of the face, dizziness, tachycardia, and throbbing headache. Large doses cause vomiting, restlessness, blurred vision, hypotension (which can be severe), syncope, and rarely cyanosis, and methaemoglobinaemia; impairment of respiration and bradycardia may ensue.

PRECAUTIONS / WARNINGS:

Isosorbide Dinitrate should not be used in patients with severe hypotension, hypovolaemia, marked anaemia, constrictive pericarditis, or raised intracranial pressure due to head trauma or cerebral haemorrhage. It may increase intraocular pressure in patients with closed-angle glaucoma and it is recommended by some that it should be avoided in such patients.

It should be used with caution in patients with severely impaired renal or hepatic function, hypothyroidism, malnutrition, or hypothermia. Also care is required in patients with early myocardial infarction. Some effects of this drug are enhanced by alcohol.

Development of tolerance may occur with all forms of nitrate therapy particularly with the long acting preparations that maintain continuously high plasma nitrate concentration.

Use in pregnancy and lactation:

It should be used during pregnancy only if the potential benefit justifies the potential risk to the foetus and caution

should be exercised when administered to a nursing mother.

DRUG INTERACTIONS:

The following drug interactions and / or related problems have been selected on the basis of their potential clinical significance (possible mechanism in parentheses where appropriate – not necessarily inclusive)

Note: Combinations containing any of the following medications, depending on the amount present may also interact with this medication.

Acetylcholine or Histamine or Norepinephrine (levarterenol) - (Effects may be decreased when used concurrently with nitrates)

Alcohol, moderate or excessive amounts or Antihypertensives or

Hypotension-producing medications, other or Opioid (narcotic) analgesics or Vasodilators, other – (Concurrent use may intensify the orthostatic hypotensive effects of nitrates; dosage adjustments may be necessary)

Sympathomimetics – (Concurrent use may reduce the antianginal effects of nitrates, which may counteract the pressor effect of sympathomimetics possibly resulting in hypotension)

Alcohol may enhance any marked sensitivity to the hypotensive effect of nitrates.

RECOMMENDED DOSAGE, DOSAGE SCHEDULE & ROUTE OF ADMINISTRATION:

Usual adult dose: Antianginal – Oral, 5 to 20 mg every six hours, dosage being adjusted as needed and tolerated.

The dosage range is 5 to 40 mg four times a day, with the usual dosage range being 20 to 40 mg four times a day.

Usual pediatric dose: Dosage has not been established.

SYMPTOMS & TREATMENTS FOR OVERDOSAGE, AND ANTIDOTE (S):

For treatment of overdose

Syncope and hypotension should be treated by keeping the patient in a recumbent position with the head lowered.

The rapid metabolism of nitroglycerin usually makes

